



This internal document is meant to act as a quick reference for the core claims used within any branded material on a global level. It does not take into account local regulatory adjustments or content outside of the main brand story.

The claims contained in this document are not intended to be used in isolation. The global team is responsible for the substantiation of the claims, and LOC should not re-substantiate. Each LOC is responsible for ensuring claims are consistent with local legal, regulatory, and industry code requirements and that all claims are communicated with appropriate context, including applicable safety information, to ensure a fair, accurate, and balanced presentation.

All claims in each section grouped by colour must be used together. Claims with no colour must follow any red text instructions or may be used standalone if not otherwise specified.

Red text is for instructional purposes, including business rules and context around claims, and is not part of the claim itself.



Table of Contents

CAMPAIGN/COVER3

PATIENT PROFILES4

ENDOMETRIAL CANCER UNMET NEED/BIOMARKERS.....5

RUBY PART 1 TRIAL DESIGN6

EFFICACY RESULTS.....9

SUBSEQUENT IMMUNOTHERAPY.....19

SAFETY PROFILE22

DOSING AND ADMINISTRATION.....25

SUMMARY MESSAGES.....29

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
CAMPAIGN/COVER			
Let their light SHINE longer with JEMPERLI (dostarlimab)			
Let their light <i>Shine</i> longer with JEMPERLI (dostarlimab) JEMPERLI + CP: The first and only IO combination with a statistically significant OS benefit in all-comers with primary advanced or recurrent endometrial cancer^{1-7*} INDICATION JEMPERLI is indicated in combination with carboplatin and paclitaxel for the first-line treatment of adult patients with primary advanced or recurrent endometrial cancer (EC) and who are candidates for systemic therapy. *All-comers OS analysis: Median OS with JEMPERLI + CP was 44.6 months vs 28.2 months with CP alone (HR=0.69; 95% CI: 0.54, 0.89; P=0.002). Kaplan-Meier probability of survival at 24 months was 70.1% (95% CI: 63.8, 75.5) with JEMPERLI + CP vs 54.3% (95% CI: 47.8, 60.3) with CP alone.^{1,7} PM-QBL-DST-EDTL-240002 Date of preparation: January 2025 For healthcare professionals only. Not an actual patient taking JEMPERLI.			
JEMPERLI + CP: The first and only IO combination with a statistically significant OS benefit in all-comers with primary advanced or recurrent endometrial cancer*	*All-comers OS analysis: Median OS with JEMPERLI + CP was 44.6 months vs 28.2 months with CP alone (HR=0.69; 95% CI: 0.54, 0.89; P=0.002). Kaplan-Meier probability of survival at 24 months was 70.1% (95% CI: 63.8, 75.5) with JEMPERLI + CP vs 54.3% (95% CI: 47.8, 60.3) with CP alone.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Eskander RN, et al. <i>N Engl J Med.</i> 2023;388(23):2159-2170. Westin SN, et al; on behalf of the DUO-E Investigators. <i>J Clin Oncol.</i> 2023;42(3):283-299. Keytruda. Summary of Product Characteristics. Merck & Co, Inc. 2025. Imfinzi. Summary of Product Characteristics. AstraZeneca Pharmaceuticals LP. 2025. Baurain J-F, et al. Society of Gynecologic Oncology 2024 Annual Meeting on Women’s Cancer. Presented 16 March 2024. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	“First and only” subhead is a claim that requires supporting data in the footnote to be displayed with the subhead.

JEMPERLI Global Branded ITT Core Claims Document



CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
INDICATION JEMPERLI is indicated in combination with carboplatin and paclitaxel for the first-line treatment of adult patients with primary advanced or recurrent endometrial cancer (EC) and who are candidates for systemic therapy.		JEMPERLI Summary of Product Characteristics. GSK. 2025.	RUBY Part 1 indication may be used on the cover page of materials that only present RUBY Part 1 data throughout.

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
PATIENT PROFILES			
When dMMR/MSI-H Endometrial Cancer Returns, Offer the Proven Combination of JEMPERLI + CP		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	Individual patient profile content must be used together for each case study.
- Endometrioid - Recurrent disease (at 7 months follow-up) - dMMR/MSI-H - Underwent surgery at diagnosis and completed adjuvant chemotherapy and radiotherapy 7 months ago - PET/CT showed metastases to lung and liver at follow-up - ECOG PS 1 (at baseline)		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	
For Your Patients With Primary Advanced MMRp/MSS Endometrial Cancer, Offer JEMPERLI + CP		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	Individual patient profile content must be used together for each case study.
- Endometrioid - Stage IVB disease - MMRp/MSS - Currently recovering from surgery following diagnosis and has not yet received any systemic therapy - PET/CT scan showed lung metastasis - ECOG PS 1 (at baseline)		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	
For Primary Advanced Endometrial Cancer, Start With the Proven Combination of JEMPERLI + CP		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	Individual patient profile content must be used together for each case study.
- Carcinosarcoma - Stage IIIA disease - MMR/MSI status unknown - Currently recovering from surgery following diagnosis and has not yet received any systemic therapy - Measurable disease but PET/CT showed no involvement of nearby lymph nodes or distant sites - ECOG PS 1 (at baseline)		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	

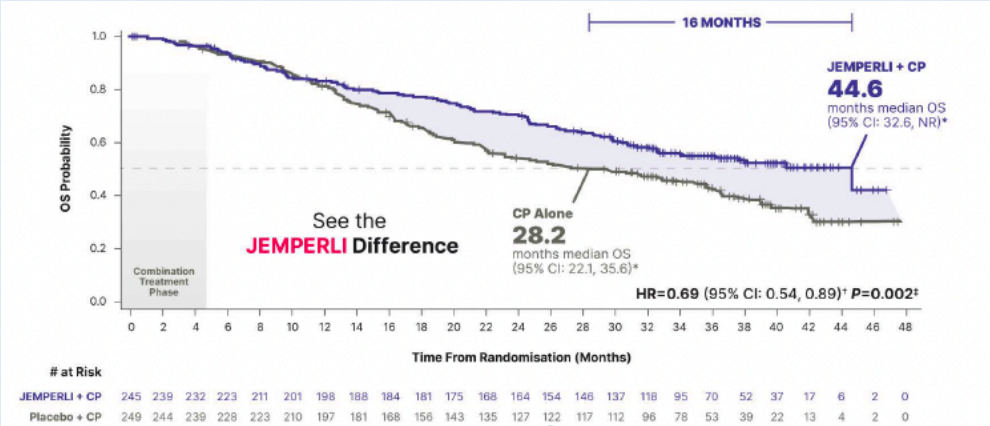
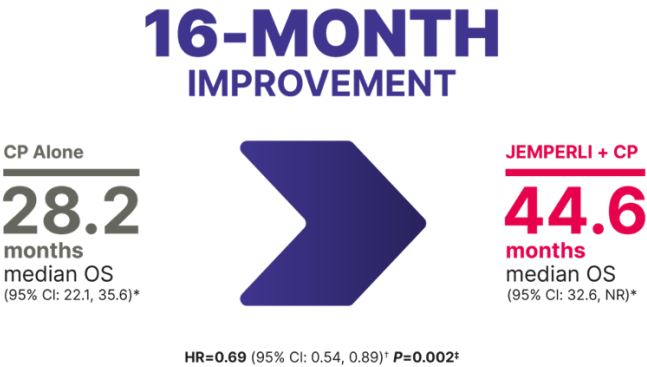
CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
ENDOMETRIAL CANCER UNMET NEED/BIOMARKERS			
Optimising Patient Outcomes Starts With First-Line Treatment Selection		Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	Headline must be supported by following three content blocks.
<3 Years median survival rate for patients with primary advanced or recurrent EC receiving standard-of-care CP alone		Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	
40% of endometrial cancer deaths are attributed to aggressive non-endometrioid histologies		Bogani G, et al. <i>Int J Gynecol Cancer.</i> 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol.</i> 2019;37(22):1895-1908.	
~70% to 75% of endometrial tumours have MMRp/MSS biomarker status and have a higher proportion of aggressive histologies than those with dMMR/MSI-H		Makker V, et al. <i>Nat Rev Dis Primers.</i> 2021;7(1):88. Lorenzi M, et al. <i>J Oncol.</i> 2020;2020:1-17. doi:10.1155/2020/1807929. Bonneville R, et al. <i>JCO Precis Oncol.</i> 2017;2017:PO.17.00073. doi:10.1200/PO.17.00073 Jumaah AS, et al. <i>J Pathol Transl Med.</i> 2021;55:202-211.	
ESMO guidelines recommend biomarker testing for all endometrial cancer patients.		Oaknin A, et al. <i>Ann Oncol.</i> 2022;33(9):860-877.	
Testing provides important information about your patients			
dMMR endometrial cancer tumours can be associated with: - a higher rate of recurrence - a higher rate of distant recurrences - inherited mutations which may be associated with Lynch syndrome (5% of cases); a condition characterised by increased risk of certain types of cancers		Backes FJ, et al. <i>Cancer.</i> 2019;125(3):398-405. Evrard C, et al. <i>Cancers. (Basel).</i> 2021;13(10):2434.	
Modalities to test for dMMR/MSI-H: - IHC - dMMR - Loss of ≥1 MMR protein - PCR/NGS - MSI-H - PCR: DNA repeats are unstable - NGS: Altered loci		Evrard C, et al. <i>Cancers. (Basel).</i> 2021;13(10):2434. Vanderwalde A, et al. <i>Cancer Med J.</i> 2018;7(3):746-756.	

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
ESMO guidelines recommend that patients with MMRp tumours receive other biomarker testing.		Oaknin A, et al. <i>Ann Oncol.</i> 2022;33(9):860-877.	

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
RUBY PART 1 TRIAL DESIGN			
<i>In All-Comers With Primary Advanced or Recurrent EC</i> At 3+ Years, JEMPERLI + CP Has the Longest Median Follow-up for an Immunotherapy Combination to Date*	*The median duration of follow-up, defined as time from randomisation to data cutoff, was 37.2 months (cutoff date 22 September 2023).	JEMPERLI Summary of Product Characteristics. GSK. 2025. Eskander RN, et al. <i>N Engl J Med.</i> 2023;388(23):2159-2170. Westin SN, et al; on behalf of the DUO-E Investigators. <i>J Clin Oncol.</i> 2023;42(3):283-299. Keytruda. Summary of Product Characteristics. Merck & Co, Inc. 2025. Imfinzi. Summary of Product Characteristics. AstraZeneca Pharmaceuticals LP. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	[CONTEXT NOTE: Study population and tumor type = primary advanced or recurrent EC; study name = RUBY Part 1] Reference numbers within this image are from the source image. If using this image elsewhere, please use the references noted in this document. Because of this, the asset number in the visuals are in a manner to which you can customise as needed.
	†Randomisation was stratified by MMR/MSI status, prior external pelvic radiotherapy, and disease status (recurrent, primary Stage III, or primary Stage IV). ‡Treatment continued for up to 3 years or until unacceptable toxicity, disease progression, or investigator decision. §PFS assessed by the investigator according to RECIST v1.1.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	
RUBY Part 1 Trial Design: A phase 3, randomised, double-blind trial of patients with primary advanced or recurrent EC (N=494, all-comers) who were randomised 1:1 to JEMPERLI + CP or placebo + CP Q3W for 6 cycles, followed by JEMPERLI or placebo Q6W, respectively, until disease progression, unacceptable toxicity, or up to 3 years. Major efficacy endpoints were investigator-assessed PFS by RECIST v1.1 in the dMMR/MSI-H and all-comers populations, and OS in all-comers		JEMPERLI Summary of Product Characteristics. GSK. 2025.	Study design language is mandatory to include with efficacy content. Do not have to include this trial design language if using graphic above.

CLAIM/GRAPHIC		FOOTNOTE	REFERENCE	BUSINESS RULES	
See how the RUBY Part 1 patient population aligns with real-world disease characteristics, including aggressive histologies			JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. Bogani G, et al. <i>Int J Gynecol Cancer.</i> 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol.</i> 2019;37(22):1895-1908.	[CONTEXT NOTE: This CTA language is driving to key eligibility criteria and patient characteristics]	
The RUBY Part 1 patient population aligns with real-world disease characteristics, including aggressive histologies			JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. Bogani G, et al. <i>Int J Gynecol Cancer.</i> 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol.</i> 2019;37(22):1895-1908.		
In Primary Advanced or Recurrent EC RUBY Part 1 Included Patients With Broad Disease Characteristics			JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	[CONTEXT NOTE: Key eligibility criteria headline]	
Primary FIGO Stage III or Stage IV disease, including patients with more aggressive histologies such as carcinosarcoma and serous adenocarcinoma			JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. Bogani G, et al. <i>Int J Gynecol Cancer.</i> 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol.</i> 2019;37(22):1895-1908.	[CONTEXT NOTE: All primary copy should be visually treated as the same bullet-level copy.] Reference numbers within this image are from the source image. If using this image elsewhere, please use the references noted in this document. Because of this, the asset number in the visuals are in a manner that you can customise as needed.	
Measurable Disease¹*	Measurable* or Non-Measurable Disease¹		*Measurable or evaluable by RECIST v1.1.		JEMPERLI Summary of Product Characteristics. GSK. 2025.
Stage IIIA-IIIIC1	Stage IIIIC1 patients with carcinosarcoma, clear cell, serous, or mixed histology (≥10% carcinosarcoma, clear cell, or serous histology)				
			</		

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
All patients were anti–PD-1/L1/L2 naïve		ClinicalTrials.gov. Accessed 18 November 2024. http://clinicaltrials.gov/study/NCT03981796	
Prior radiation was not permitted within 21 days of study treatment excluding palliative radiotherapy, which was permitted within up to 1 week of study treatment		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
In Primary Advanced or Recurrent EC RUBY Part 1 (N=494) Included Patients With Aggressive Histologies		JEMPERLI Summary of Product Characteristics. GSK. 2025. Bogani G, et al. <i>Int J Gynecol Cancer</i> . 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol</i> . 2019;37(22):1895-1908.	[CONTEXT NOTE: Patient characteristics headline]
Age 51% 65 Years or Older Race 77% White 12% Black 3% Asian		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Biomarkers 24% dMMR/MSI-H 76% MMRp/MSS		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Histologies 9% Carcinosarcoma 21% Serous carcinoma 15% All other histologies Clear-cell adenocarcinoma, mucinous adenocarcinoma, undifferentiated carcinoma, mixed carcinoma (≥10% of carcinosarcoma, clear-cell, or serous histologic type) or other 55% Endometrioid carcinoma		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med</i> . 2023;388(23):2145-2158.	
Disease Status 19% Primary FIGO Stage III 34% Primary FIGO Stage IV 48% Recurrent Disease		JEMPERLI Summary of Product Characteristics. GSK. 2025.	

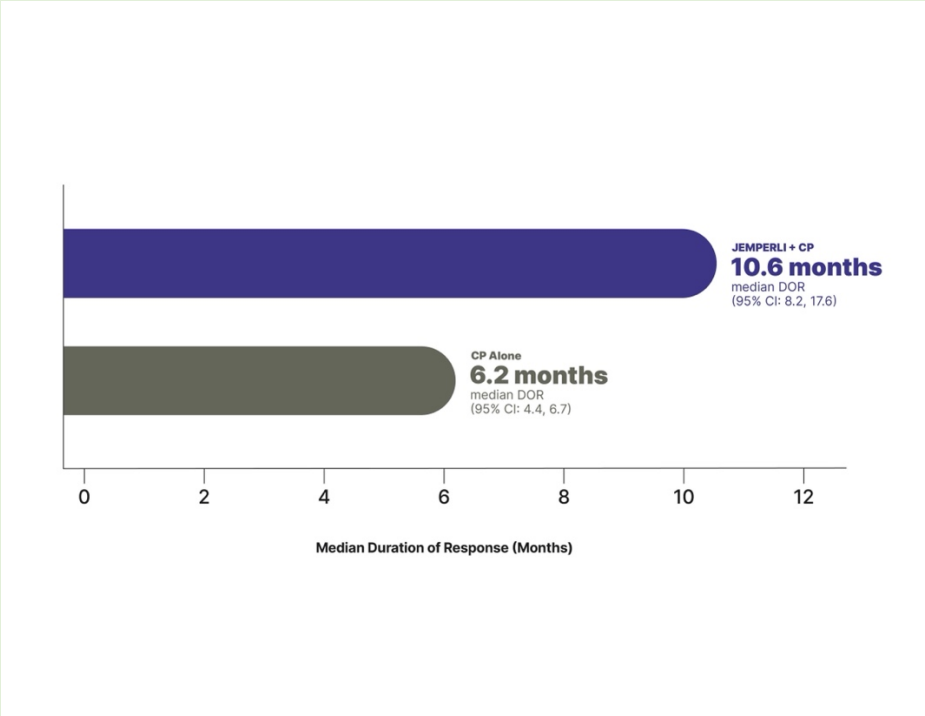
CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
EFFICACY RESULTS			
BUSINESS RULES Please ensure that efficacy information is presented with the study design when implemented in promotional materials. Non-primary efficacy outcome measures (dMMR/MSI-H OS, MMRp/MSS OS/PFS, ORR, and DOR) should not be presented in isolation without covering primary efficacy outcome measures (all-comers OS/PFS, dMMR/MSI-H PFS). DOR describes the duration of response for patients with a partial or complete response and should therefore not be presented without also covering the response rates (ORR). Patient population must be clear for any efficacy claim used.			
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] Primary Efficacy Outcome Measure 16-Month Improvement in Median Overall Survival vs CP alone		JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	
Statistically significant 31% reduction in the risk of death with JEMPERLI + CP vs CP alone 	Data cutoff 22 September 2023. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model. ‡One-sided <i>P</i> -value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] 	Data cutoff 22 September 2023. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model. ‡One-sided <i>P</i> -value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	All-comers OS efficacy alt graphic can be used in addition to the KM curve above OR in secondary pieces as a reminder if KM curve has already been presented.

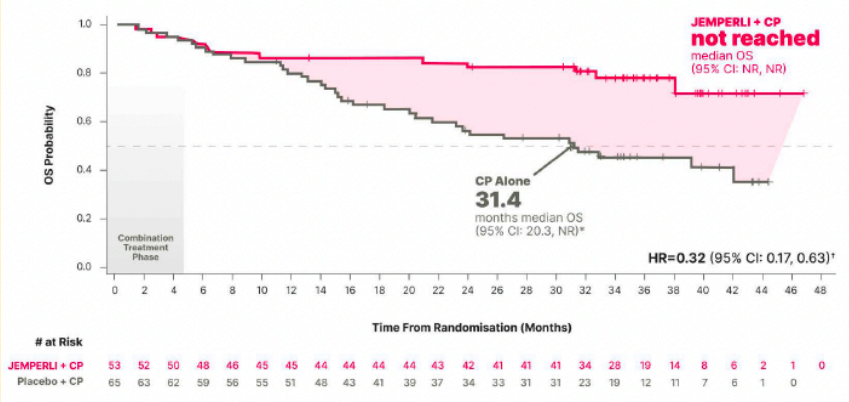
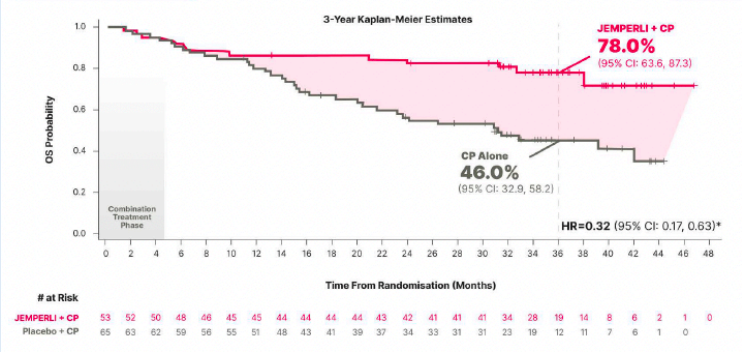
JEMPERLI Global Branded ITT Core Claims Document



CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES																																																				
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] All-comers median OS was 44.6 months (95% CI: 32.6, NR)* with JEMPERLI + CP vs 28.2 months (95% CI: 22.1, 35.6)* with CP alone (HR=0.69; 95% CI: 0.54, 0.89†; P<0.002‡)	Data cutoff 22 September 2023. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model. ‡One-sided P-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	All-comers OS efficacy alt text can be used in instances where KM curve above is not.																																																				
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] 3-Year Kaplan-Meier Estimates JEMPERLI + CP 54.9% (95% CI: 48.2, 61.2) CP Alone 42.9% (95% CI: 36.3, 49.3) HR=0.69 (95% CI: 0.54, 0.89)* P=0.002† Combination Treatment Phase # at Risk <table><tr><td>JEMPERLI + CP</td><td>245</td><td>239</td><td>232</td><td>223</td><td>211</td><td>201</td><td>198</td><td>188</td><td>184</td><td>181</td><td>175</td><td>168</td><td>164</td><td>154</td><td>146</td><td>137</td><td>118</td><td>95</td><td>70</td><td>52</td><td>37</td><td>17</td><td>6</td><td>2</td><td>0</td></tr><tr><td>Placebo + CP</td><td>249</td><td>244</td><td>239</td><td>228</td><td>223</td><td>210</td><td>197</td><td>181</td><td>168</td><td>156</td><td>143</td><td>135</td><td>127</td><td>122</td><td>117</td><td>112</td><td>96</td><td>78</td><td>53</td><td>39</td><td>22</td><td>13</td><td>4</td><td>2</td><td>0</td></tr></table>	JEMPERLI + CP	245	239	232	223	211	201	198	188	184	181	175	168	164	154	146	137	118	95	70	52	37	17	6	2	0	Placebo + CP	249	244	239	228	223	210	197	181	168	156	143	135	127	122	117	112	96	78	53	39	22	13	4	2	0	Data cutoff 22 September 2023. *Based on stratified Cox regression model. †One-sided P-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	[CONTEXT NOTE: 3-year Kaplan-Meier estimate KM curve in all-comers] 3-year Kaplan-Meier curve graphic is the same curve as shown in row above for median OS, but with different callouts. Only medians and 95% CI are required to be shown with the 3-year KM curve. 3-year callouts are optional but must be used in conjunction with median OS callouts shown previously.
JEMPERLI + CP	245	239	232	223	211	201	198	188	184	181	175	168	164	154	146	137	118	95	70	52	37	17	6	2	0																														
Placebo + CP	249	244	239	228	223	210	197	181	168	156	143	135	127	122	117	112	96	78	53	39	22	13	4	2	0																														
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] Estimated Kaplan-Meier probability of survival at 3 years was 54.9% (95% CI: 48.2, 61.2) with JEMPERLI + CP vs 42.9% (95% CI: 36.3, 49.3) with CP alone (HR=0.69; 95% CI: 0.54, 0.89*; P=0.002†)	Data cutoff 22 September 2023. *Based on stratified Cox regression model. †One-sided P-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	Data that provides estimates at specific time points is required to travel with full KM curves.																																																				
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] Primary Efficacy Outcome Measure ~1-Year Median PFS in a Population That Included Those With Aggressive Histologies	Data cutoff 28 September 2022.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. Bogani G, et al. <i>Int J Gynecol Cancer.</i> 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol.</i> 2019;37(22):1895-1908.	[CONTEXT NOTE: All-comers progression-free survival efficacy results]																																																				

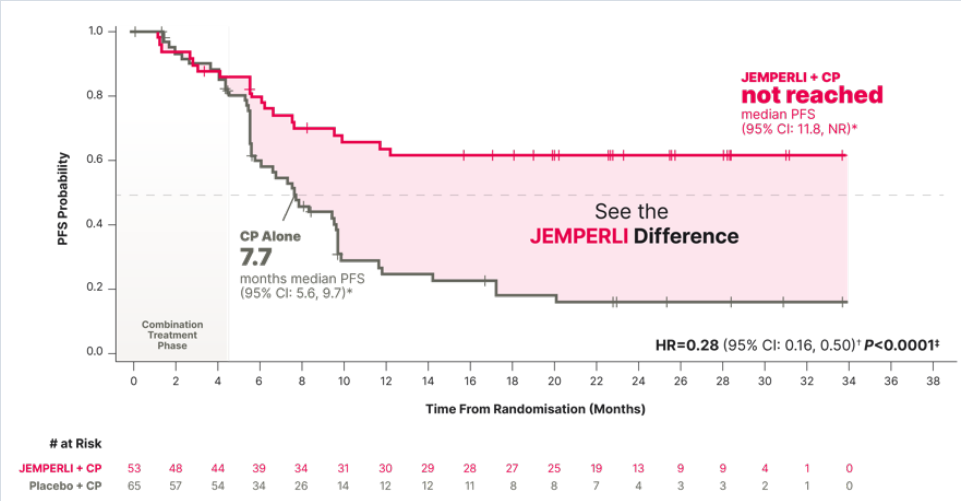
CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES																																								
Statistically significant 36% reduction in the risk of progression or death with JEMPERLI + CP vs CP alone # at Risk <table><tr><td>JEMPERLI + CP</td><td>245</td><td>220</td><td>197</td><td>157</td><td>130</td><td>105</td><td>94</td><td>90</td><td>84</td><td>78</td><td>66</td><td>52</td><td>34</td><td>23</td><td>22</td><td>12</td><td>2</td><td>0</td></tr><tr><td>Placebo + CP</td><td>249</td><td>219</td><td>200</td><td>144</td><td>103</td><td>74</td><td>59</td><td>57</td><td>48</td><td>42</td><td>39</td><td>32</td><td>20</td><td>14</td><td>13</td><td>5</td><td>2</td><td>1</td><td>1</td><td>0</td></tr></table>	JEMPERLI + CP	245	220	197	157	130	105	94	90	84	78	66	52	34	23	22	12	2	0	Placebo + CP	249	219	200	144	103	74	59	57	48	42	39	32	20	14	13	5	2	1	1	0	Data cutoff 28 September 2022. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model. ‡One-sided <i>P</i>-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. Bogani G, et al. <i>Int J Gynecol Cancer.</i> 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol.</i> 2019;37(22):1895-1908.	
JEMPERLI + CP	245	220	197	157	130	105	94	90	84	78	66	52	34	23	22	12	2	0																									
Placebo + CP	249	219	200	144	103	74	59	57	48	42	39	32	20	14	13	5	2	1	1	0																							
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] All-comers median PFS was 11.8 months (95% CI: 9.6, 17.1)* with JEMPERLI + CP vs 7.9 months (95% CI: 7.6, 9.5)* with CP alone (HR=0.64; 95% CI: 0.51, 0.80†; <i>P</i><0.0001‡)	Data cutoff 28 September 2022. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model. ‡One-sided <i>P</i>-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	All-comers PFS efficacy alt text can be used in instances where KM curve above is not.																																								
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] JEMPERLI + CP: ~1-year median PFS in a population that included those with aggressive histologies - Statistically significant 36% reduction in the risk of progression or death with JEMPERLI + CP vs CP alone (HR=0.64; 95% CI: 0.51, 0.80*; <i>P</i><0.0001†) 11.8 months median PFS (95% CI: 9.6, 17.1)‡ with JEMPERLI + CP vs 7.9 months (95% CI: 7.6, 9.5)‡ with CP alone	Data cutoff 28 September 2022. *Based on stratified Cox regression model. †One-sided <i>P</i>-value based on stratified log-rank test. ‡By Brookmeyer and Crowley method.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. Bogani G, et al. <i>Int J Gynecol Cancer.</i> 2023;33(2):147-174. Clarke MA, et al. <i>J Clin Oncol.</i> 2019;37(22):1895-1908.	All-comers PFS efficacy alt text can be used in instances where KM curve above is not.																																								

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] <i>Additional Efficacy Outcome Measures In the All-Comers Population</i> Patients Achieved a Durable Response with JEMPERLI + CP Median DOR of nearly 11 months with JEMPERLI + CP*† Median DOR in patients with primary advanced or recurrent endometrial cancer 	Data cutoff 28 September 2022. Median duration of follow-up of 25.4 months. *Assessed by investigator according to RECIST v1.1. †For patients with a partial or complete response.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	[CONTEXT NOTE: Objective response rate and duration of response in all-comers measured in patients with evaluable disease at baseline]
38.0% probability of remaining in response at 2 years (95% CI: 29.4, 46.5) with JEMPERLI + CP vs 13.0% (95% CI: 7.5, 20.2) with CP alone‡	‡Probability of response was estimated from Kaplan-Meier curves.	Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	
More patients responded to JEMPERLI + CP* 70.3% of patients with evaluable disease at baseline in the all-comers population responded with JEMPERLI + CP (149/212) 95% CI: 63.6, 76.3 - CR=25.0%; PR=45.3% 64.8% of patients with evaluable disease at baseline responded with CP alone (142/219) 95% CI: 58.1, 71.2 - CR=19.6%; PR=45.2	*Assessed by investigator according to RECIST v1.1.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	
[Patient population: dMMR/MSI-H subgroup] <i>Exploratory Analysis</i> Median OS Was Not Reached With JEMPERLI + CP vs 31.4 Months With CP Alone		JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
68% reduction in the risk of death observed with JEMPERLI + CP vs CP alone  The prespecified exploratory analysis for OS was not powered to detect treatment differences; results are descriptive	Data cutoff 22 September 2023. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	[CONTEXT NOTE: Median OS in the dMMR/MSI-H subgroup KM curve] Disclaimer must accompany claims for exploratory endpoints.
[Patient population: dMMR/MSI-H subgroup] Exploratory Analysis Median OS Was Not Reached With JEMPERLI + CP vs 31.4 Months With CP Alone		JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	
68% reduction in the risk of death observed with JEMPERLI + CP vs CP alone  The prespecified exploratory analysis for OS was not powered to detect treatment differences; results are descriptive	Data cutoff 22 September 2023. *Based on stratified Cox regression model.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	[CONTEXT NOTE: 3-year Kaplan-Meier estimate KM curve in dMMR/MSI-H subgroup] Disclaimer must accompany claims for exploratory endpoints. 3-year Kaplan-Meier curve graphic is the same curve as shown in row above for median OS, but with different callouts. Only medians and 95% CI are required to be shown with the KM curve. 3-year callouts are optional but must be used in conjunction with median OS callouts shown previously.

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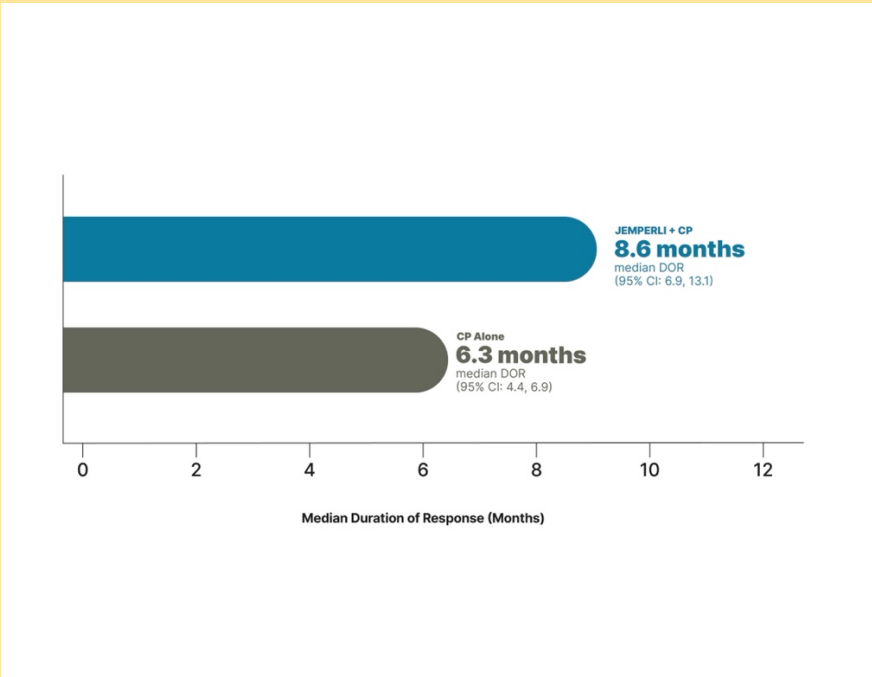


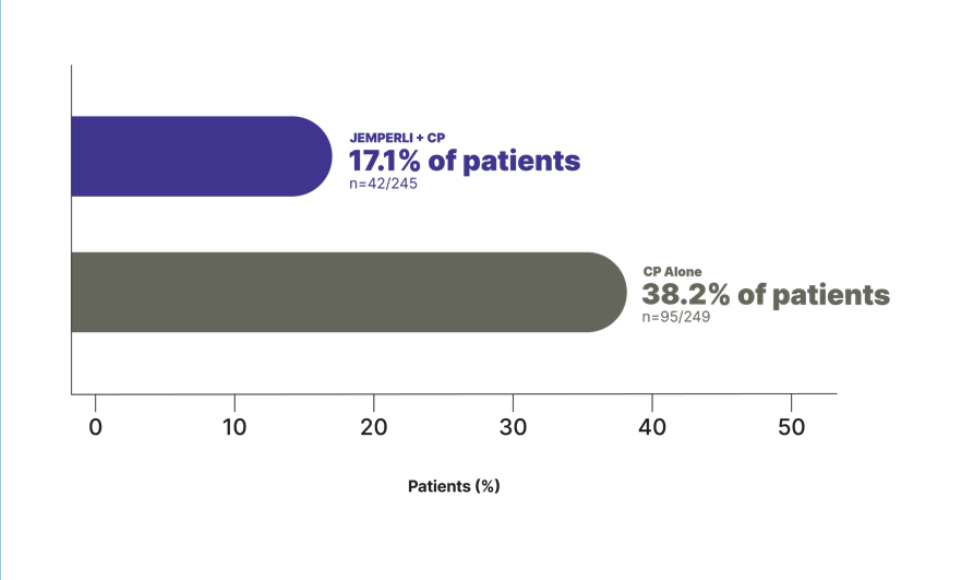
CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
[Patient population: dMMR/MSI-H subgroup] Estimated Kaplan-Meier probability of survival at 3 years was 78.0% (95% CI: 63.6, 87.3) with JEMPERLI + CP vs 46.0% (95% CI: 32.9, 58.2) with CP alone (HR=0.32; 95% CI: 0.17, 0.63)*	Data cutoff 22 September 2023. *Based on stratified Cox regression model.	Powell MA, et al. <i>Ann Oncol</i>. 2024;35(8):728-738. JEMPERLI Summary of Product Characteristics. GSK. 2025.	Data that provides estimates at specific time points is required to travel with full KM curves.
[Patient population: dMMR/MSI-H subgroup] <i>Primary Efficacy Outcome Measure</i> Median PFS Was Not Reached With JEMPERLI + CP After >2 Years of Follow-up Compared With 7.7 Months With CP Alone	Data cutoff 28 September 2022.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	[CONTEXT NOTE: dMMR/MSI-H subgroup progression-free survival efficacy results]
Statistically significant 72% reduction in the risk of progression or death vs CP alone 	Data cutoff 28 September 2022. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model. ‡One-sided <i>P</i>-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med</i>. 2023;388(23):2145-2158.	
[Patient population: dMMR/MSI-H subgroup] dMMR/MSI-H subgroup median PFS (a primary efficacy endpoint) was not reached (95% CI: 11.8, NR)* with JEMPERLI + CP after >2 years of follow-up compared with 7.7 months (95% CI: 5.6, 9.7)* with CP alone (HR=0.28; 95% CI: 0.16, 0.50†; <i>P</i><0.0001‡)	Data cutoff 28 September 2022. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model. ‡One-sided <i>P</i>-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	dMMR/MSI-H PFS efficacy alt text can be used in instances where KM curve above is not.
[Patient population: dMMR/MSI-H subgroup] Median PFS (a primary efficacy endpoint) was not reached (95% CI: 11.8, NR)* with JEMPERLI + CP after >2 years follow-up compared with 7.7 months (95% CI: 5.6, 9.7)* with CP alone Statistically significant 72% reduction in the risk of progression or death vs CP alone (HR=0.28; 95% CI: 0.16, 0.50†; <i>P</i><0.0001‡)	Data cutoff 28 September 2022. *By Brookmeyer and Crowley method. †Based on stratified Cox regression model.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	dMMR/MSI-H PFS efficacy alt text can be used in instances where KM curve above is not.

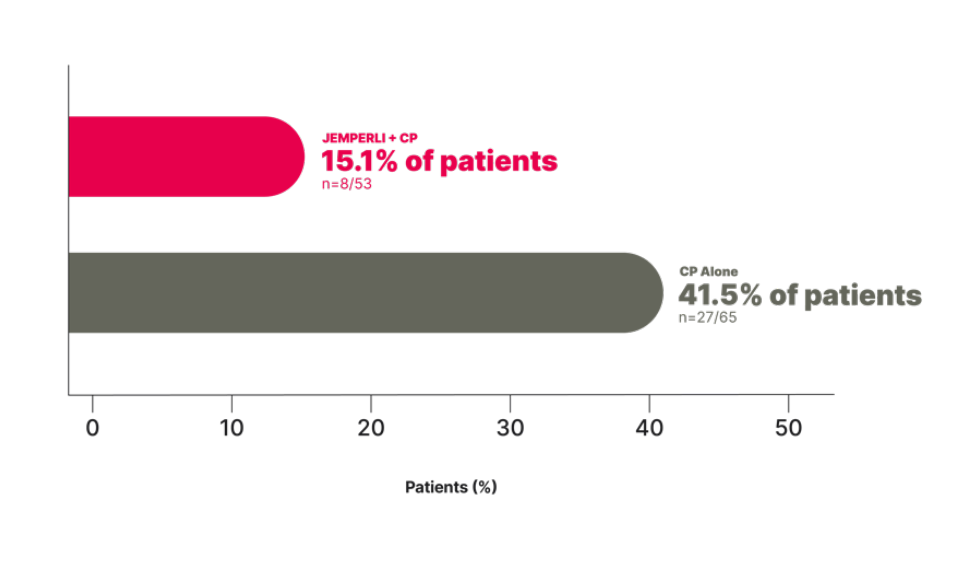
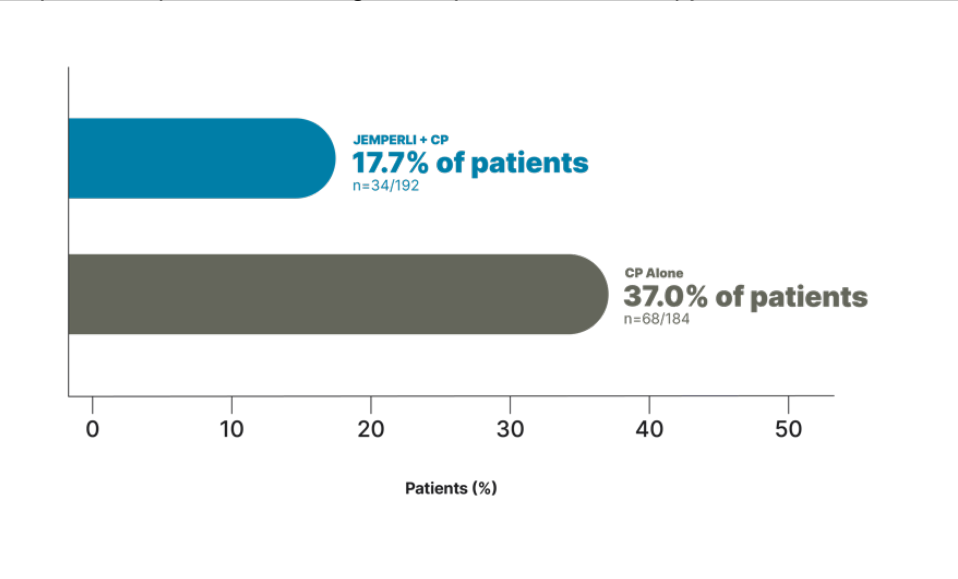
CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES									
	†One-sided <i>P</i> -value based on stratified log-rank test.											
[Patient population: dMMR/MSI-H subgroup] <i>Additional Efficacy Outcome Measures In the dMMR/MSI-H Subgroup</i> Patients Achieved a Durable Response with JEMPERLI + CP Median DOR was not reached at 2+ years*† Median DOR in patients with dMMR/MSI-H primary advanced or recurrent endometrial cancer <table><caption>Median Duration of Response (Months)</caption><tr><th>Group</th><th>Median DOR (Months)</th><th>95% CI (Months)</th></tr><tr><td>CP Alone</td><td>5.4</td><td>3.9, 8.1</td></tr><tr><td>JEMPERLI + CP</td><td>not reached</td><td>10.1, NE</td></tr></table>	Group	Median DOR (Months)	95% CI (Months)	CP Alone	5.4	3.9, 8.1	JEMPERLI + CP	not reached	10.1, NE	Data cutoff 28 September 2022. Median duration of follow-up of 24.8 months. *Assessed by investigator according to RECIST v1.1. †For patients with a partial or complete response.	Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. JEMPERLI Summary of Product Characteristics. GSK. 2025.	[CONTEXT NOTE: Objective response rate and duration of response in dMMR/MSI-H subgroup measured in patients with evaluable disease at baseline]
Group	Median DOR (Months)	95% CI (Months)										
CP Alone	5.4	3.9, 8.1										
JEMPERLI + CP	not reached	10.1, NE										
62.1% probability of remaining in response at 2 years (95% CI: 44.4, 75.5) with JEMPERLI + CP vs 13.2% (95% CI: 4.6, 26.3) with CP alone†	†Probability of response was estimated from Kaplan-Meier curves.	Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.										
More patients responded to JEMPERLI + CP* 77.6% of patients with evaluable disease at baseline in the dMMR/MSI-H subgroup responded with JEMPERLI + CP (38/49) 95% CI: 63.4, 88.2 - CR=30.6%; PR=46.9%	*Assessed by investigator according to RECIST v1.1.	Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158. JEMPERLI Summary of Product Characteristics. GSK. 2025.										

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
69.0% of patients with evaluable disease at baseline responded with CP alone (40/58) 95% CI: 55.5, 80.5 - CR=20.7%; PR=48.3%			
[Patient population: MMRp/MSS subgroup] <i>Exploratory Analysis</i> 7-month Clinically Meaningful Improvement in Median OS Observed With JEMPERLI + CP OS Probability Time From Randomisation (Months) # at Risk JEMPERLI + CP 192 187 182 175 165 156 153 144 140 137 131 125 122 113 105 96 84 67 51 38 29 11 4 1 0 Placebo + CP 184 181 177 169 167 155 146 133 125 115 104 98 93 89 86 81 73 59 41 28 15 7 3 2 0 CP Alone 27.0 months median OS (95% CI: 21.5, 35.6) JEMPERLI + CP 34.0 months median OS (95% CI: 28.6, NE) HR=0.79 (95% CI: 0.60, 1.04) 7 MONTHS Combination Treatment Phase The prespecified exploratory analysis for OS was not powered to detect treatment differences; results are descriptive.	Data cutoff 22 September 2023.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738. Ellis LM, et al. <i>J Clin Oncol.</i> 2014;32(12):1277-1280. Powell MA, et al. Society of Gynecologic Oncology 2024 Annual Meeting on Women’s Cancer. Presented 16 March 2024.	[CONTEXT NOTE: Median OS in the MMRp/MSS subgroup KM curve] Disclaimer must accompany claims for exploratory endpoints.
[Patient population: MMRp/MSS subgroup] <i>Exploratory Analysis</i> 7-month Clinically Meaningful Improvement in Median OS Observed With JEMPERLI + CP OS Probability Time From Randomisation (Months) # at Risk JEMPERLI + CP 192 187 182 175 165 156 153 144 140 137 131 125 122 113 105 96 84 67 51 38 29 11 4 1 0 Placebo + CP 184 181 177 169 167 155 146 133 125 115 104 98 93 89 86 81 73 59 41 28 15 7 3 2 0 CP Alone 41.9% (95% CI: 34.3, 49.4) JEMPERLI + CP 48.6% (95% CI: 41.0, 55.7) HR=0.79 (95% CI: 0.60, 1.04) 3-Year Kaplan-Meier Estimates Combination Treatment Phase The prespecified exploratory analysis for OS was not powered to detect treatment differences; results are descriptive.	Data cutoff 22 September 2023.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738. Ellis LM, et al. <i>J Clin Oncol.</i> 2014;32(12):1277-1280.	[CONTEXT NOTE: 3-year Kaplan-Meier estimate KM curve in MMRp/MSS subgroup] Disclaimer must accompany claims for exploratory endpoints. 3-year Kaplan-Meier curve graphic is the same curve as shown in row above for median OS, but with different callouts. Only medians and 95% CI are required to be shown with the KM curve. 3-year callouts are optional but must be used in conjunction with median OS callouts shown previously.

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
[Patient population: MMRp/MSS subgroup] Estimated Kaplan-Meier probability of survival at 3 years was 48.6% (95% CI: 41.0, 55.7) with JEMPERLI + CP vs 41.9% (95% CI: 34.3, 49.4) with CP alone (HR=0.79; 95% CI: 0.60, 1.04)	Data cutoff 22 September 2023.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol</i>. 2024;35(8):728-738.	Data that provides estimates at specific time points is required to travel with full KM curves.
[Patient population: MMRp/MSS subgroup] <i>Exploratory Analysis</i> 24% Reduction in the Risk of Progression or Death Observed vs CP Alone The prespecified exploratory analysis for PFS was not powered to detect treatment differences; results are descriptive.	Data cutoff 28 September 2022.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med</i>. 2023;388(23):2145-2158. Mirza MR, et al. Society of Gynecologic Oncology 2023 Annual Meeting on Women’s Cancer. Presented 27 March 2023.	[CONTEXT NOTE: MMRp/MSS subgroup progression-free survival efficacy results] Disclaimer must accompany claims for exploratory endpoints.
[Patient population: MMRp/MSS subgroup] MMRp/MSS subgroup median PFS was 9.9 months (95% CI: 9.0, 13.3) with JEMPERLI + CP vs 7.9 months (95% CI: 7.6, 9.8) with CP alone (HR=0.76; 95% CI: 0.59, 0.98) The prespecified exploratory analysis for PFS was not powered to detect treatment differences; results are descriptive.	Data cutoff 28 September 2022.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med</i>. 2023;388(23):2145-2158. Mirza MR, et al. Society of Gynecologic Oncology 2023 Annual Meeting on Women’s Cancer. Presented 27 March 2023.	Disclaimer must accompany claims for exploratory endpoints. “Exploratory PFS analysis of MMRp/MSS” alt text can be used in instances where KM curve above is not.

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
[Patient population: MMRp/MSS subgroup] <i>Additional Efficacy Outcome Measures In the MMRp/MSS Subgroup</i> Patients Achieved a Durable Response with JEMPERLI + CP Median DOR of nearly 9 months with JEMPERLI + CP^{††} Median DOR in patients with MMRp/MSS primary advanced or recurrent endometrial cancer 	Data cutoff 28 September 2022. *Analysis of DOR in the MMRp/MSS population was post hoc. †Assessed by investigator according to RECIST v1.1. ‡For patients with a partial or complete response.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	[CONTEXT NOTE: Objective response rate and duration of response in MMRp/MSS subgroup measured in patients with evaluable disease at baseline]
28.4% probability of remaining in response at 2 years (95% CI: 19.1, 38.4) with JEMPERLI + CP vs 13.5% (95% CI: 7.1, 22.0) with CP alone[§]	§Probability of response was estimated from Kaplan-Meier curves.	Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	
More patients responded to JEMPERLI + CP[†] 68.1% of patients with evaluable disease at baseline in the MMRp/MSS subgroup responded with JEMPERLI + CP (111/163) 95% CI: 60.4, 75.2 - CR=23.3%; PR=44.8% 63.4% of patients with evaluable disease at baseline responded with CP alone (102/161) 95% CI: 55.4, 70.8 - CR=19.3%; PR=44.1%	†Assessed by investigator according to RECIST v1.1.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
SUBSEQUENT IMMUNOTHERAPY			
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] <i>In RUBY Part 1</i> More Than Double the Amount of Patients Treated With CP Alone Went on to Receive Subsequent Immunotherapy vs Those Receiving JEMPERLI + CP		Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	
Proportion of patients receiving subsequent immunotherapy in RUBY Part 1  After study treatment discontinuation, patients were permitted to receive any follow-up anticancer therapy at the clinician’s discretion.		Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	
[Patient population: All-comers (overall population of patients with primary advanced or recurrent EC)] More than double the amount of patients treated with CP alone (38.2%; n=95/249) went on to receive subsequent immunotherapy vs those receiving JEMPERLI + CP (17.1%; n=42/245)*	*After study treatment discontinuation, patients were permitted to receive any follow-up anticancer therapy at the clinician’s discretion.	Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	[USAGE NOTE: All-comers subsequent IO alt text Can be used in instances where graphic above is not]
[Patient population: dMMR/MSI-H subgroup] <i>In RUBY Part 1</i> More Than Double the Amount of Patients Treated With CP Alone Went on to Receive Subsequent Immunotherapy vs Those Receiving JEMPERLI + CP		Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	[CONTEXT NOTE: Subsequent immunotherapies in dMMR/MSI-H subgroup]

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
Proportion of patients receiving subsequent immunotherapy in RUBY Part 1  After study treatment discontinuation, patients were permitted to receive any follow-up anticancer therapy at the clinician’s discretion.		Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	
[Patient population: dMMR/MSI-H subgroup] More than double the amount of patients treated with CP alone (41.5%; n=27/65) went on to receive subsequent immunotherapy vs those receiving JEMPERLI + CP (15.1%; n=8/53)*	*After study treatment discontinuation, patients were permitted to receive any follow-up anticancer therapy at the clinician’s discretion.	Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	dMMR/MSI-H IO alt text can be used in instances where graphic above is not.
[Patient population: MMRp/MSS subgroup] In RUBY Part 1 Double the Amount of Patients Treated With CP Alone Went on to Receive Subsequent Immunotherapy vs Those Receiving JEMPERLI + CP Proportion of patients receiving subsequent immunotherapy in RUBY Part 1 		Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	[CONTEXT NOTE: Subsequent immunotherapies in MMRp/MSS subgroup]

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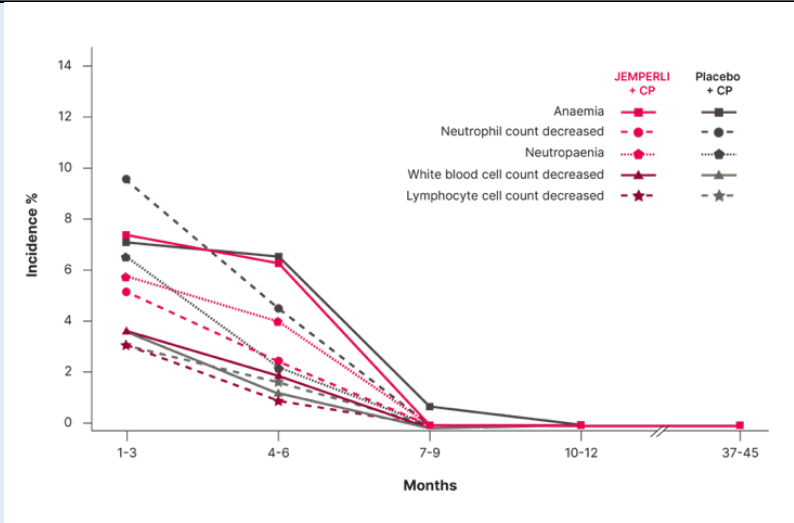
CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
After study treatment discontinuation, patients were permitted to receive any follow-up anticancer therapy at the clinician’s discretion.			
[Patient population: MMRp/MSS subgroup] Double the amount of patients treated with CP alone (37.0%; n=68/184) went on to receive subsequent immunotherapy vs those receiving JEMPERLI + CP (17.7%; n=34/192)*	*After study treatment discontinuation, patients were permitted to receive any follow-up anticancer therapy at the clinician’s discretion.	Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	MMRp/MSS subsequent IO alt text can be used in instances where graphic above is not.

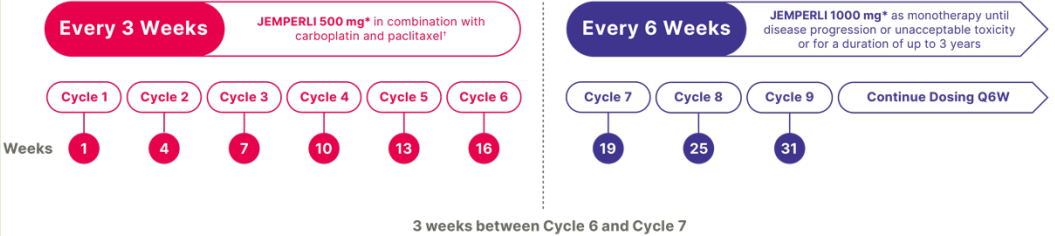


CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
SAFETY PROFILE			
<i>In Primary Advanced or Recurrent EC</i> JEMPERLI + CP Exhibited a Manageable Safety Profile		JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738.	Language may be adjusted depending on LOC requirements, for example, adding the word “generally” before the word “manageable.”
Adverse reactions in RUBY Part 1 - JEMPERLI was permanently discontinued due to adverse reactions in 12 (5.0%) patients; most were immune-related events - In patients with primary advanced or recurrent EC receiving JEMPERLI + CP (N=241), the most common adverse reactions (≥10%) were rash (23.2%), rash maculopapular (14.5%), hypothyroidism (14.5%), pyrexia (12.9%), alanine aminotransferase increased (12.9%), aspartate aminotransferase increased (12.0%), and dry skin (10.0%) - Adverse reactions were serious in 5.8% of patients. The most frequent (>1%) serious adverse reaction was pyrexia (2.9%) - The most frequent (>10%) immune-related adverse reaction (irAR) was hypothyroidism (12.0%), with maculopapular rash (1.2%) the most frequent (>1%) irAR leading to treatment discontinuation		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
- Early identification and management of irARs are essential to ensure safe use of PD-1/PD-L1 blocking antibodies - Patients should be monitored for signs and symptoms of immune- and infusion-related adverse reactions. Haematological and clinical chemistries, including liver, kidney, and thyroid function tests, should be evaluated at baseline and periodically during treatment - Based on the severity and type of irARs, treatment with JEMPERLI should be withheld or permanently discontinued and appropriate treatment administered		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Treatment-emergent adverse events (TEAEs) >30% in either arm in RUBY Part 1 Most common undesirable events which occurred or worsened once treatment was started in RUBY Part 1	TEAEs were through 90 days post last dose of treatment or until the start of another anticancer therapy.	Powell MA, et al. <i>Ann Oncol.</i> 2024;35(8):728-738. Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	



CLAIM/GRAPHIC			FOOTNOTE	REFERENCE	BUSINESS RULES	
Adverse Event	JEMPERLI + CP (N=241)	Placebo + CP (N=246)				
	no. of patients (%)	no. of patients (%)				
	Nausea	131 (54.4%)				114 (46.3%)
	Alopecia	130 (53.9%)				123 (50.0%)
	Fatigue	126 (52.3%)				135 (54.9%)
	Neuropathy, peripheral	106 (44.0%)				103 (41.9%)
	Anaemia	91 (37.8%)				105 (42.7%)
	Arthralgia	90 (37.3%)				87 (35.4%)
	Constipation	84 (34.9%)				89 (36.2%)
	Diarrhoea	76 (31.5%)				72 (29.3%)
An adverse event (AE) is any untoward (not favourable) medical occurrence in a patient, or a clinical trial participant receiving a medicine, and which does not necessarily have a causal relationship with this treatment. An adverse reaction (AR) is any adverse event or experience related to a medicine for which a reasonable causal relationship with the medicine's use is suspected. A treatment-emergent adverse event (TEAE) is an adverse event that occurs or worsens after treatment has started, through 90 days after the last dose of study treatment.				EUPATI Open Classroom. Glossary. Utrecht (NL): EUPATI; Accessed 10 December 2024. https://toolbox.eupati.eu/glossary	Mirza MR, et al. <i>N Engl J Med.</i> 2023;388(23):2145-2158.	Definition footnotes can be included with safety content or at the end of material.
The 5 most common Grade ≥3 treatment-related AEs (TRAEs) were observed during the first 3 to 6 months of RUBY Part 1				Lokich E, et al. American Society of Clinical Oncology (ASCO) 2024 Annual Meeting. Presented 31 May 2024.	[CONTEXT NOTE: CTA driving to TRAE rates]	REF-251218 Auranen 2024 is a published article that may be used to support TRAE claims in LOCs that may not use the Lokich poster.
In RUBY Part 1 The 5 Most Common Grade ≥3 TRAEs Were Observed During the First 3-6 Months				Lokich E, et al. American Society of Clinical Oncology (ASCO) 2024 Annual Meeting. Presented 31 May 2024.	[CONTEXT NOTE: TRAE rates]	
Time to onset for most common Grade ≥3 TRAEs				JEMPERLI Summary of Product Characteristics. GSK. 2025.	Lokich E, et al. American Society of Clinical Oncology (ASCO) 2024 Annual Meeting. Presented 31 May 2024.	

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
			
Patients received CP in combination with JEMPERLI during the first ~4 months of RUBY Part 1 (Q3W for 6 cycles)		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
A treatment-related adverse event is an AE that was classified by study investigators as related to treatment.		Auranen A, et al. <i>Ther Adv Med Oncol.</i> 2024;16:1-17.	Definition footnote can be included with safety content or at the end of material.

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
DOSING AND ADMINISTRATION			
The JEMPERLI dosing regimen reduces the frequency of dosing visits for your patients after transitioning from Q3W to Q6W dosing		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
JEMPERLI + CP Dosing Established in RUBY Part 1 Deliver a Proven Regimen: Combination Upfront, Then JEMPERLI Monotherapy		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Recommended dosage of JEMPERLI in primary advanced or recurrent endometrial cancer 	*30-minute intravenous infusion. †First 6 cycles are administered in combination with carboplatin and paclitaxel. Refer to the Summary of Product Characteristics for the agents administered in combination with JEMPERLI, as appropriate. Administer JEMPERLI prior to carboplatin and paclitaxel on the same day.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	
- The Q3W dosing schedule allows for more frequent patient monitoring during the 6-cycle treatment initiation phase - The number of infusion visits is reduced after transitioning to the Q6W monotherapy phase - Additional monitoring may be required per clinical discretion		JEMPERLI Summary of Product Characteristics. GSK. 2025.	

CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
Recommended Dosage of JEMPERLI in Combination With Carboplatin and Paclitaxel in Primary Advanced or Recurrent Endometrial Cancer The diagram illustrates the recommended dosing schedule for JEMPERLI in combination with carboplatin and paclitaxel for primary advanced or recurrent endometrial cancer. It is divided into two main sections: 'Every 3 Weeks' and 'Every 6 Weeks'. The 'Every 3 Weeks' section shows Cycles 1 through 6, with JEMPERLI 500 mg* administered in combination with carboplatin and paclitaxel. The 'Every 6 Weeks' section shows Cycles 7 through 9, with JEMPERLI 1000 mg* administered as monotherapy until disease progression or unacceptable toxicity, or for a duration of up to 3 years. A gap of 3 weeks is indicated between Cycle 6 and Cycle 7. The diagram also includes a 'Continue Dosing Q6W' instruction.	*First 6 cycles are administered in combination with carboplatin and paclitaxel. Refer to the Summary of Product Characteristics for the agents administered in combination with JEMPERLI, as appropriate. Administer JEMPERLI prior to carboplatin and paclitaxel on the same day.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	Alt text graphic with horizontal layout that may be used instead of dosing graphic above.
Recommended Dosage Modifications		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Management of Adverse Reactions* Dose reduction is not recommended. Dosing delay or discontinuation may be required based on individual safety and tolerability. Recommended modifications to manage adverse reactions are summarised below.	*Toxicity graded per National Cancer Institute Common Terminology Criteria for Adverse Events (CTCAE) version 5.0.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	

CLAIM/GRAPHIC			FOOTNOTE	REFERENCE	BUSINESS RULES
Adverse Reaction	Severity Grade*	Dose Modification	*Toxicity graded per National Cancer Institute Common Terminology Criteria for Adverse Events (CTCAE) version 5.0. †For patients with liver metastases who begin treatment with grade 2 increase of AST or ALT, if AST or ALT increases by ≥50% relative to baseline and lasts for at least 1 week, then treatment should be discontinued.	JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Immune-Related Adverse Reactions					
Colitis	Grade 2 or 3	Withhold dose. Restart dosing when toxicity resolves to Grade 0 or 1.			
	Grade 4	Permanently discontinue.			
Hepatitis	Grade 2 with AST or ALT >3 and up to 5 × ULN or total bilirubin >1.5 and up to 3 × ULN	Withhold dose. Restart dosing when toxicity resolves to Grade 0 or 1.			
	Grade ≥3 with AST or ALT >5 × ULN or total bilirubin >3 × ULN	Permanently discontinue (see exception below).†			
Type 1 diabetes mellitus (T1DM)	Grade 3 or 4 (hyperglycaemia)	Withhold dose. Restart dosing in appropriately managed, clinically and metabolically stable patients.			
Hypophysitis or adrenal insufficiency	Grade 2, 3 or 4	Withhold dose. Restart dosing when toxicity resolves to Grade 0 or 1. Permanently discontinue for recurrence or worsening while on adequate hormonal therapy.			
Hypothyroidism or hyperthyroidism	Grade 3 or 4	Withhold dose. Restart dosing when toxicity resolves to Grade 0 or 1.			
Pneumonitis	Grade 2	Withhold dose. Restart dosing when toxicity resolves to Grade 0 or 1. If Grade 2 recurs, permanently discontinue.			
	Grade 3 or 4	Permanently discontinue.			
Nephritis	Grade 2	Withhold dose. Restart dosing when toxicity resolves to Grade 0 or 1.			
	Grade 3 or 4	Permanently discontinue.			
Exfoliative dermatologic conditions (e.g. SJS, TEN, DRESS)	Suspected	Withhold dose for any Grade. Restart dosing if not confirmed and when toxicity resolves to Grade 0 or 1.			
	Confirmed	Permanently discontinue.			
Myocarditis	Grade 2, 3 or 4	Permanently discontinue.			
Severe neurological toxicities (myasthenic syndrome/myasthenia gravis, Guillain-Barré syndrome, encephalitis, transverse myelitis)	Grade 2, 3 or 4	Permanently discontinue.			
Other immune-related adverse reactions (including but not limited to myositis, sarcoidosis, autoimmune haemolytic anaemia, pancreatitis, iridocyclitis, uveitis, diabetic ketoacidosis, arthralgia, solid organ transplant rejection, graft-versus-host disease)	Grade 3	Withhold dose. Restart dosing when toxicity resolves to Grade 0 or 1.			
	Grade 4	Permanently discontinue.			
Recurrence of immune-related adverse reactions after resolution to ≤Grade 1 (except for pneumonitis, see above)	Grade 3 or 4	Permanently discontinue.			
Other Adverse Reactions					
Infusion-related reactions	Grade 2	Withhold dose. If resolved within 1 hour of stopping, may be restarted at 50% of the original infusion rate, or restart when symptoms resolve with pre-medication. If Grade 2 recurs with adequate premedication, permanently discontinue.			
	Grade 3 or 4	Permanently discontinue.			

JEMPERLI Global Branded ITT Core Claims Document



CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
Preparation/Dilution, Storage and Administration			
Parenteral medicinal products should be inspected visually for particulate matter and discolouration prior to administration. JEMPERLI is a slightly opalescent colourless to yellow solution. Discard the vial if visible particles are observed		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
JEMPERLI is compatible with an IV bag made of polyvinyl chloride (PVC) with or without di(2-ethylhexyl) phthalate (DEHP), ethylene vinyl acetate, polyethylene (PE), polypropylene (PP) or polyolefin blend (PP+PE), and a syringe made from PP		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
JEMPERLI Is Available in Fixed-Dose Vials		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
For the 500-mg dose Withdraw 10 mL of JEMPERLI from a vial and transfer into an IV bag containing: - Sodium chloride 9 mg/mL (0.9%) solution for injection or - Glucose 50 mg/mL (5%) solution for injection The final concentration of the diluted solution should be between 2 mg/mL and 10 mg/mL. The total volume of the infusion solution must not exceed 250 mL. This may require withdrawing a volume of diluent from the IV bag prior to adding a volume of JEMPERLI into the IV bag.		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
For the 1000-mg dose Withdraw 10 mL of JEMPERLI from each of two vials (withdraw 20 mL total) and transfer into an IV bag containing: - Sodium chloride 9 mg/mL (0.9%) solution for injection or - Glucose 50 mg/mL (5%) solution for injection The final concentration of the diluted solution should be between 4 mg/mL and 10 mg/mL. The total volume of the infusion solution must not exceed 250 mL. This may require withdrawing a volume of diluent from the IV bag prior to adding a volume of JEMPERLI into the IV bag.		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Mix diluted solution by gentle inversion. Do not shake the final infusion bag. Discard any unused portion left in the vial		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Storage Do not use this medicine after the expiry date, which is stated on the carton and vial label after EXP. The expiry date refers to the last day of that month. Store the vial under refrigeration (2 °C-8 °C) in the original carton until time of preparation in order to protect from light. Do not freeze. The prepared dose may be stored either: - At room temperature up to 25 °C for no more than 6 hours from the time of dilution until the end of infusion - Under refrigeration at 2 °C to 8 °C for no more than 24 hours from the time of dilution until end of infusion. If refrigerated, allow the diluted solution to come to room temperature prior to administration		JEMPERLI Summary of Product Characteristics. GSK. 2025.	
Administration JEMPERLI should be administered by IV infusion using an IV infusion pump over 30 minutes by a health care practitioner. JEMPERLI must not be administered as an IV push or bolus injection. Do not co-administer other medicinal products through the same infusion line. Tubing should be made of PVC, platinum cured silicon or PP; fittings made from PVC or polycarbonate and needles made from stainless steel. A 0.2- or 0.22-micron in-line polyethersulfone (PES) filter must be used during administration of JEMPERLI. Any unused medicinal product or waste material should be disposed of in accordance with local requirements.		JEMPERLI Summary of Product Characteristics. GSK. 2025.	



CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
SUMMARY MESSAGES			
<i>In Patients With Primary Advanced or Recurrent EC</i> JEMPERLI + CP: The First and Only IO Combination With a Statistically Significant OS Benefit in All-Comers		JEMPERLI Summary of Product Characteristics. GSK. 2025. Eskander RN, et al. <i>N Engl J Med</i>. 2023;388(23):2159-2170. Westin SN, et al; on behalf of the DUO-E Investigators. <i>J Clin Oncol</i>. 2023;42(3):283-299. Keytruda. Summary of Product Characteristics. Merck & Co, Inc. 2025. Imfinzi. Summary of Product Characteristics. AstraZeneca Pharmaceuticals LP. 2025. Baurain J-F, et al. Society of Gynecologic Oncology 2024 Annual Meeting on Women’s Cancer. Presented 16 March 2024. Powell MA, et al. <i>Ann Oncol</i>. 2024;35(8):728-738.	Summary messages that can be used at the end of a piece after full data is presented.
16-month improvement in median OS for all-comers vs CP alone Significant OS benefit with HR 0.69 (95% CI: 0.54, 0.89*; <i>P</i>=0.002†)	*Based on stratified Cox regression model. †One-sided <i>P</i>-value based on stratified log-rank test.	JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol</i>. 2024;35(8):728-738.	
<u>Prespecified exploratory analysis in patients with dMMR/MSI-H EC</u>: Median OS was not reached with JEMPERLI + CP vs 31.4 months with CP alone with 3+ years median follow-up		JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol</i>. 2024;35(8):728-738.	
<u>Prespecified exploratory analysis in patients with MMRp/MSS EC</u>: 7-month improvement in median OS was seen with JEMPERLI + CP		JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol</i>. 2024;35(8):728-738.	
Robust trial design including patients with aggressive histologies and longest median follow-up to date for an IO combination (3+ years)		JEMPERLI Summary of Product Characteristics. GSK. 2025. Mirza MR, et al. <i>N Engl J Med</i>. 2023;388(23):2145-2158. Bogani G, et al. <i>Int J Gynecol Cancer</i>. 2023;33(2):147-174.	



CLAIM/GRAPHIC	FOOTNOTE	REFERENCE	BUSINESS RULES
		Clarke MA, et al. <i>J Clin Oncol</i> . 2019;37(22):1895-1908. Powell MA, et al. <i>Ann Oncol</i> . 2024;35(8):728-738.	
Manageable safety profile - JEMPERLI was permanently discontinued due to adverse reactions in 12 (5.0%) patients - The most common adverse reactions (≥10%) were rash (23.2%), rash maculopapular (14.5%), hypothyroidism (14.5%), pyrexia (12.9%), alanine aminotransferase increased (12.9%), aspartate aminotransferase increased (12.0%), and dry skin (10.0%)		JEMPERLI Summary of Product Characteristics. GSK. 2025. Powell MA, et al. <i>Ann Oncol</i> . 2024;35(8):728-738.	
[USAGE NOTE: LOCs must define all acronyms/abbreviations if they are using the core claims as found within this document.] GLOSSARY OF ACRONYMS AE=adverse event; ALT=alanine aminotransferase; AST=aspartate aminotransferase; CI=confidence interval; CP=carboplatin + paclitaxel; CR=complete response; CT=computed tomography; dMMR=mismatch repair deficient; DOR=duration of response; DRESS=drug reaction with eosinophilia and systemic symptoms; EC=endometrial cancer; ECOG=Eastern Cooperative Oncology Group; ESMO=European Society for Medical Oncology; FIGO=International Federation of Gynecology and Obstetrics; HR=hazard ratio; IHC=immunohistochemistry; IO=immuno-oncology; irAR=immune-related adverse event; IV=intravenous; MMR=mismatch repair; MMRp=mismatch repair proficient; MSI=microsatellite instability; MSI-H=microsatellite instability-high; MSS=microsatellite stable; NE=not estimable; NGS=next-generation sequencing; NR=not reached; ORR=objective response rate; OS=overall survival; PCR=polymerase chain reaction; PD-1=programmed death receptor 1; PD-L1/L2=programmed death ligand 1/2; PET=positron emission tomography; PFS=progression-free survival; PR=partial response; PS=performance status; Q3W=every 3 weeks; Q6W=every 6 weeks; RECIST v1.1=Response Evaluation Criteria in Solid Tumors v1.1; SJS=Stevens-Johnson syndrome; TEAE=treatment-emergent adverse event; TEN=toxic epidermal necrolysis; TRAE=treatment-related adverse event; ULN=upper limit of normal.			

Electronic Certificate

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Role	Signature
Jim Reimer - Commercial (jim.x.reimer@gsk.com)	It is approved that this material has been examined and is believed to be in accordance with the relevant Code of Practice and any other relevant regulations, policies and SOPs. Date: 13-May-2025 11:12:07 GMT+0000
Odette Allonby - Medical (odette.x.allonby@gsk.com)	It is approved that this material has been examined and is believed to be in accordance with the relevant Code of Practice and any other relevant regulations, policies and SOPs. Date: 16-May-2025 14:05:05 GMT+0000